2023 CT Problem Gambling Prevalence Survey: Methods and Key Results

Rachel A. Volberg, PhD
Gemini Research, Inc.
Northampton, MA
USA

May 14, 2024

Disclosure of Potential Conflict of Interest

- The SEIGMA study is funded by the Massachusetts Gaming Commission (MGC) and based at the University of Massachusetts Amherst.
- I also run a private consulting company, Gemini Research, that specializes in gambling research.
- I have had no affiliation with the gambling industry in the past 10 years.
- In the past 10 years, I have received research funding from government agencies in Connecticut, Massachusetts, New Jersey, Oregon, and Washington State as well as Australia, Britain, Canada, New Zealand, and Sweden.
- I have received honoraria and reimbursement for travel costs to present at conferences.

Overview of the CT Impact Study

- Gemini Research responded to a competitive bid for gambling impact study required by CT Legislature
- Contract awarded to Gemini Research and study carried out Jan-August 2023
- Examined social/health and economic/fiscal impacts of gambling in CT since mid-1990s
- Included review of state's PG prevention and treatment services

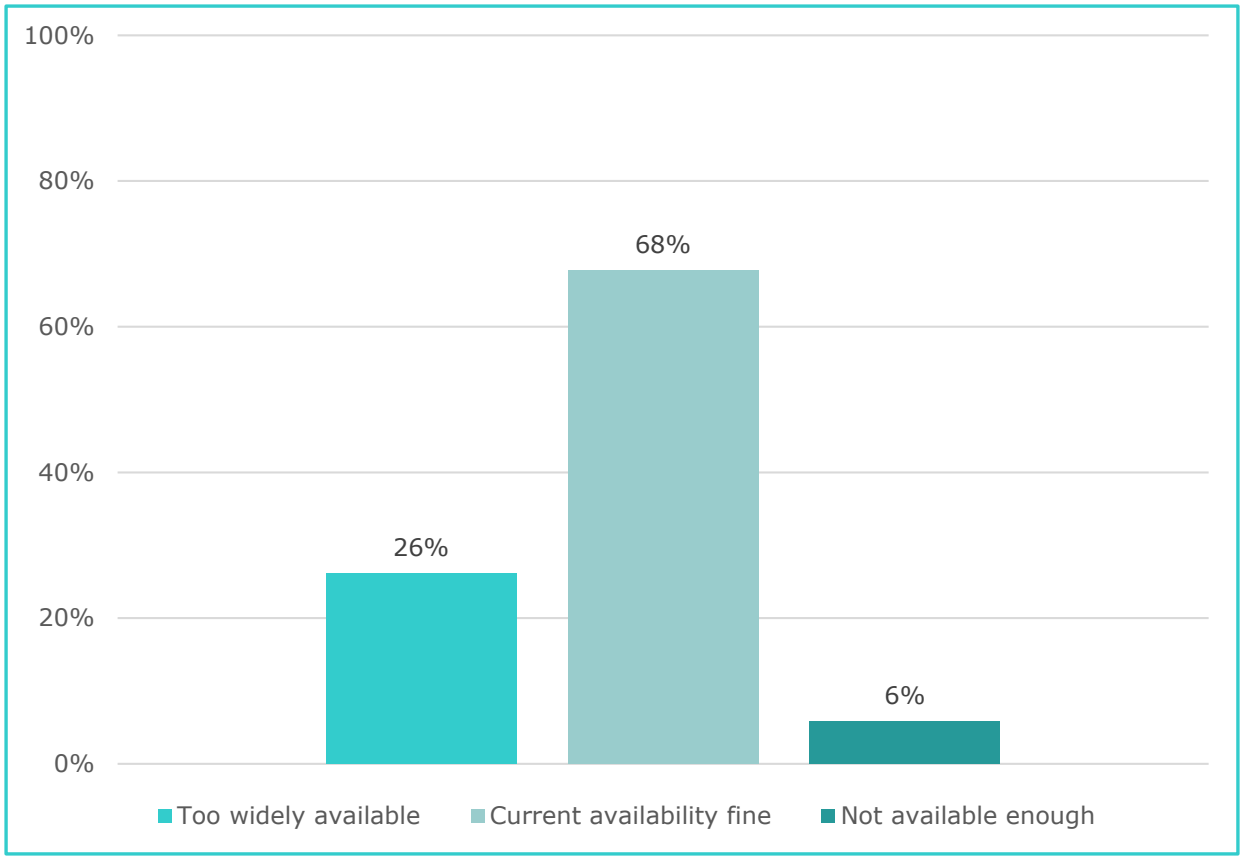
Mixed methods research strategy

- Key informant interviews (n=47)
- Population survey (n=5,239)
- Online panel survey (n=2,847)
- Secondary data on wide range of social & economic indices
- Review of prior CT population surveys and socioeconomic studies
- Data from gambling operators & CT government
- Economic modeling using REMI
- AirSage cell phone location data to determine origin of CT casino patrons
- Review of CT problem gambling services

Population Survey Process

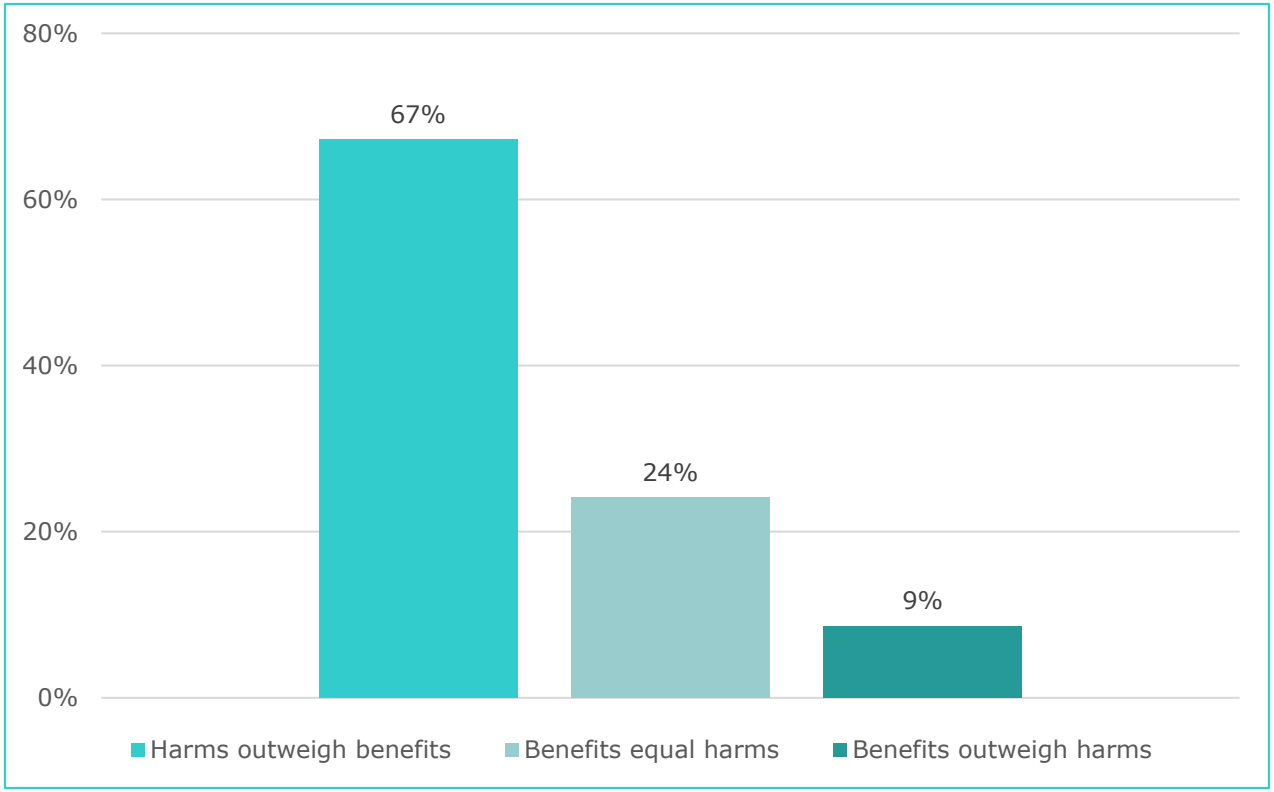
Stage	Dates	Task
1	Winter 2023	Questionnaire development & programming • Spanish translation • Computer Assisted Web Interview • Self Administered Questionnaire
2		Ethics review
3	March – April 2023	Data collection • Completed by 5,239 CT adults aged 18 and over • 12% response rate
4	May 2023	Data cleaning and weighting
5	May – June 2023	Data analysis
6	June 2023	Preliminary report to DMHAS
7	August 2023	Final report submitted to DMHAS
8	January 2024	Revised report released by DMHAS

Attitudes: Availability



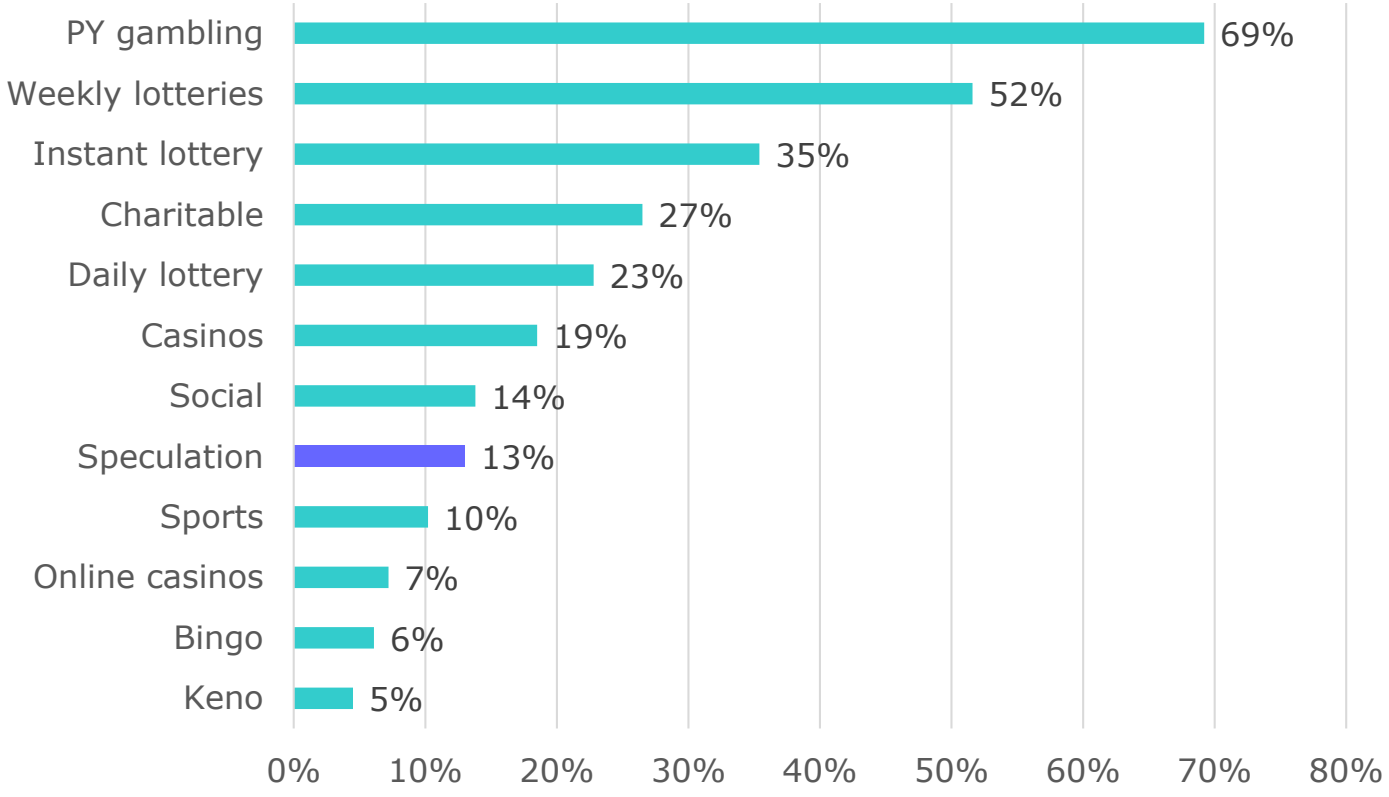
All respondents

Attitudes: Benefits and Harms



All respondents

Participation in Types of Gambling

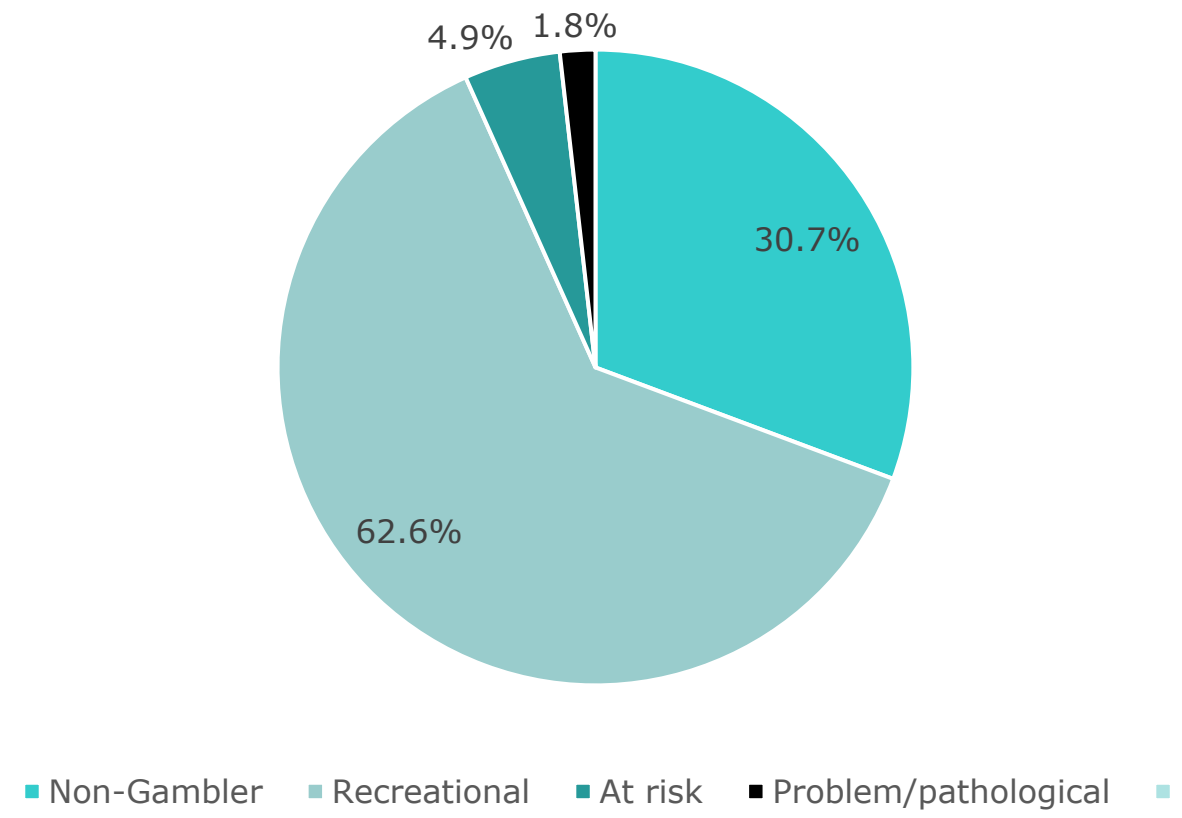


All respondents

Changes in CT Since 2008

	2008	2023
PY gambling	70.0	69.2
Any lottery product	53.7	57.1
Scratch tickets	37.0	35.4
Land-based casinos	35.6	18.5
Online gambling	2.0	10.7
Sports betting	8.4	10.2
Bingo	9.0	6.1
Horse race betting	7.4	1.7

At-risk and Problem Gambling



All respondents

Proportion of Revenue from Problem Gamblers

All lottery products	12.4%
Online casinos	21.4%
Land-based casinos	36.6%
Sports betting	51.0%
All legalized gambling	21.5%

Awareness of Services

- Everyone was asked if they had seen or heard any media or public awareness campaigns to prevent gambling problems in CT in past year
  - 52% were aware of media campaigns
- Everyone was asked if they were aware of CT problem gambling helpline
  - 52% were aware of helpline
- Aware of CT self-exclusion programs
  - 18% were aware of self-exclusion programs

Recommendations for Treatment in CT

- Problem gambling prevention and treatment services in CT provide a good model for the rest of the country
- Best practices include:
  - Age 21 requirement for casinos, sports, online gambling
  - Irrevocable self-exclusion bans
  - Availability of lifetime bans
  - Automated sports betting account freezes
- Areas of concern:
  - Total funding for services is a small fraction of $400M derived annually from problem gamblers
  - Proportion of revenue drawn from people experiencing gambling harms is too high

Specific Recommendations

- Prevention & awareness
  - Continue employing a wide array of educational & policy initiatives
  - Continue efforts to promote responsible gambling
  - Continue efforts to increase public awareness of available services
  - Continue to increase prevention work with groups at higher risk

Specific Recommendations

- Responsible gambling
  - Endeavor to reduce industry reliance on people experiencing gambling harms
  - Males, ages 35-49, non-Whites, lower educational attainment
  - Consider implementing
    - Automated alerts when gambling behavior escalates
    - Changing parameters of Reward Cards to reward RG behaviors
    - Restricting hours of service (3am – 9am)
    - Restricting ATM access or amounts
    - Better alignment of Tribes and State services

Specific Recommendations

- Voluntary self-exclusion & gambling limits
  - Merge 3 separate self-exclusion lists in CT
  - Align self-exclusion periods across lists
  - Develop regional (New England) self-exclusion program
  - Implement mandatory pre-commitment of gambling limits
- Problem gambling helpline
  - Add ‘warm hand-off’ functionality
  - Add follow-up with individual callers
  - Improve data collection & establish regular reporting schedule

Specific Recommendations

- Treatment services
  - Continue efforts to increase help-seeking
  - Improve awareness of impacts of gambling problems on other public services
  - Increase education/training for law enforcement, court officers
  - Further integration of services with MH, SU, behavioral health programs
  - Training for health professionals (inc. students) in diagnosis & referral
  - Expand bilingual treatment services
  - Require screening for gambling problems in SU/MH settings
  - Establish gambling diversion programs w/in judicial system

Specific Recommendations

- Recovery services
  - Expand availability of recovery services to prevent relapses
- Research & evaluation
  - Improve data collection on help-seeking, treatment access
  - Establish regular reporting schedule
  - Conduct online panel surveys annually
  - Add validated gambling module to BRFSS

For more information:

https://portal.ct.gov/-/media/dmhas/publications/2023-ct-final-report-jan312024.pdf